

# Standard Operating Procedure

## Writing and Issuing the Discharge Summary

«Hospital Name» — Supports AAC.13.a through AAC.13.e

|                       |                                                     |                         |                      |
|-----------------------|-----------------------------------------------------|-------------------------|----------------------|
| <b>SOP No.</b>        | [Hospital to define]                                | <b>Version</b>          | 1.0                  |
| <b>Effective Date</b> | _____                                               | <b>Review Frequency</b> | Annual, or on change |
| <b>Prepared By</b>    | Quality Coordinator Name: _____ Signature: _____    |                         |                      |
| <b>Approved By</b>    | Medical Superintendent Name: _____ Signature: _____ |                         |                      |

## 1. Purpose

This SOP describes exactly what «Hospital Name»'s discharge summary must contain, and how to write instructions the patient can actually understand and act on.

## 2. Scope

Applies to every treating doctor or doctor member of a treating team who prepares and signs a discharge or death summary.

## 3. Responsibility

**Treating Doctor:** prepares, signs, and personally explains the summary to the patient or family.

## 4. Procedure

### 4.1 Provision and acknowledgement

1. Provide the discharge summary to the patient at the time of discharge, signed by the treating doctor or a doctor member of the treating team.
2. Obtain and record the patient's or family's acknowledgement of receipt, with name and date.

### 4.2 Standardised content

1. Use the current standardised template — every summary includes, at minimum: patient's name, unique identification number, treating doctor's name, admission and discharge dates, reason for admission, significant findings, diagnosis, and treatment given.
2. If any standard field does not apply to this patient, record the reason it is not applicable rather than leaving it blank.

### 4.3 Follow-up and medication instructions

1. Write medication frequency in plain words the patient can understand — never use abbreviations such as BD, TID, or QID.
2. Include equipment and wound-care instructions where applicable.
3. Explain every instruction to the patient and/or relatives in a language and manner they understand — do not just hand over the paper. Confirm real understanding using the teach-back method (ask the patient to repeat the instruction in their own words) rather than only asking "do you understand?" — see Section 8, Reference 1.

## 4.4 Urgent-care instructions

1. Write urgent-care instructions specific to this patient's diagnosis and condition at discharge — not a generic statement.
2. Include the hospital or doctor contact number for urgent queries.

## 4.5 Death summary

1. State the cause of death clearly in the death summary.
2. Where the cause of death is unclear — including medico-legal cases — arrange and document a post-mortem.

## 5. Records

Supported by all 10 records in the AAC.13 Required Records pack.

## 6. Related Documents

- HCO\_AAC\_13\_v2\_REWRITE\_DRAFT — AAC.13 policy
- SOP — Patient Discharge Planning, Coordination and Summary Issuance (AAC.12)

## 7. Review

Reviewed annually.

## 8. References

- Agency for Healthcare Research and Quality. (2020). Health literacy universal precautions toolkit (3rd ed.) — Tool 5: Use the teach-back method.  
<https://www.ahrq.gov/health-literacy/improve/precautions/toolkit.html>